

1. Administrative & Study Identification

Full Study Title: Study of Volrustomig as Monotherapy or in Combination With Anti-Cancer Agents in Participants With Advanced/Metastatic Solid Tumors **Official Title:** A Phase II, Multi-Center Study to Evaluate the Efficacy and Safety of Volrustomig as Monotherapy or in Combination With Anti-cancer Agents in Participants With Advanced/Metastatic Solid Tumors **Short Title/Acronym:** eVOLVE-02 **Protocol Number:** D798MC00002 **NCT Number:** NCT06535607 **Posting ID:** 892150010426638666 **Sponsor Name:** AstraZeneca **Trial Status:** Recruiting **Investigational Product:** Volrustomig (MEDI5752) as Monotherapy or in Combination with Anti-Cancer Agents (Cisplatin, Carboplatin, Paclitaxel, 5-FU) **Phase of Development:** Phase 2 **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of volrustomig as monotherapy or in combination with anti-cancer agents in participants with advanced/metastatic solid tumors by assessing the Objective Response Rate (ORR) and the number of participants with adverse events/serious adverse events. **Secondary Objectives:** To evaluate the Duration of Response (DOR), Progression-Free Survival (PFS), Time to Response (TTR), and the overall safety and tolerability of the regimens.

2.2 Background & Rationale To address the unmet medical need in patients with advanced or metastatic solid tumors (including cervical cancer and head and neck squamous cell carcinoma [HNSCC]). Volrustomig, a novel bispecific antibody targeting PD-1 and CTLA-4, offers a unique approach by acting on specific pathways in cancer cells to improve targeted treatment outcomes. This study evaluates whether volrustomig, alone or combined with established chemotherapy, can enhance disease control and improve patient quality of life.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm/Open-label across multiple sub-studies/cohorts **Blinding:** Open-label **Randomization:** N/A (participants assigned to specific sub-studies based on tumor histology)

3.2 Planned Enrollment & Duration Target \$N\$: 110 evaluable participants across 3 sub-studies (30 for sub-study 1 [cervical

cancer], 20 for sub-study 2 [HNSCC monotherapy], and 60 for sub-study 3 [HNSCC combination]). **Number of Sites:** ~47 locations globally **Estimated Study Start:** 22-Aug-2024 **Estimated Primary Completion:** 30-Nov-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years at the time of signing the ICF, and body weight > 35 kg. 2. Measurable disease according to RECIST 1.1, and provision of a tumor sample to assess PD-L1 expression. 3. ECOG performance status of 0 or 1, and life expectancy ≥ 12 weeks with adequate organ/bone marrow function. 4. **Sub-study 1:** Participants with recurrent/metastatic (R/M) cervical cancer (squamous cell, adenocarcinoma, or adenosquamous histology) who have experienced disease progression during/after standard systemic therapy and received 1-2 lines of prior systemic treatment. 5. **Sub-studies 2 & 3:** Participants with incurable R/M HNSCC of the oral cavity (OC), oropharynx (OP), hypopharynx (HP), and larynx (LX). Sub-study 2 requires documented HPV status for OP cancer, and a documented PD-L1 positive result if not previously treated in the R/M setting; platinum refractory participants must have relapsed from or are refractory to prior platinum-containing regimen.

4.2 Exclusion Criteria 1. Spinal cord compression or brain metastases unless asymptomatic, stable, and not requiring steroids for at least 14 days prior to start of study intervention. 2. Primary neuroendocrine, mesenchymal, sarcomatoid histologies, or history of another primary malignancy (with exceptions for curatively treated low-risk disease ≥ 2 years ago or in situ/adequately treated nonmelanoma skin cancer). 3. Active infection (including tuberculosis, uncontrolled HIV, HBV, HCV, or active hepatitis A) or active/prior documented autoimmune/inflammatory disorders. 4. Prior exposure to any immune-mediated therapy or current/prior use of immunosuppressive medication within 14 days before the first dose. 5. Receipt of anti-cancer therapy within 28 days (or 5 half-lives) prior to first dose, wide-field radiotherapy or $> 30\%$ bone marrow radiation within 4 weeks, major surgical procedures within 4 weeks, or live attenuated vaccines within 30 days prior. 6. Not recovered ($\leq$ Grade 1 or baseline) from an adverse event due to previously administered anti-cancer therapy. 7. **Sub-study specific:** Radiotherapy within 2 weeks prior to enrollment (Sub-study 2); Contraindications to 5-FU, paclitaxel, or carboplatin (Sub-study 3).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Volrustomig administered as monotherapy or combined with anti-cancer chemotherapy (Cisplatin, Carboplatin, Paclitaxel, 5-FU) on

defined treatment cycles. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Until disease progression or unacceptable toxicity, with study evaluation averaging up to 3 years.

5.2 Concomitant & Prohibited Therapy Permitted: Palliative radiotherapy (localized), physiologic doses of oral corticosteroids (≤ 10 mg/day prednisone equivalent), and premedication for hypersensitivity reactions. **Prohibited:** Any concurrent chemotherapy except study intervention, wide-field radiotherapy within 4 weeks prior to the first dose, and live attenuated vaccines within 30 days prior.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor sample for PD-L1, Imaging (RECIST 1.1)
Baseline	Day 0	First Dose (Volrustomig +/- Chemotherapy), Vital Signs, Safety Labs
Interim	Every Cycle	Safety Labs, Efficacy Assessment (Tumor Imaging), PK/PD sampling, AE Review
End of Study	Off-treatment	Final Vital Signs, Exit Interview, Follow-up for Survival and PFS

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints

- Objective Response Rate (ORR):** Confirmed ORR, defined as the proportion of participants who have a confirmed Complete Response (CR) or Partial Response (PR), as determined by Investigator per RECIST 1.1 criteria.
- Adverse Events / Serious Adverse Events:** The number of participants with adverse events and with serious adverse events, including abnormal clinical observations, abnormal Electrocardiogram (ECG) parameters, abnormal laboratory assessments, and abnormal vital signs that changed from baseline.

7.2 Secondary & Exploratory Endpoints

- Duration of Response (DOR):** Measured from the time of initial response until documented tumor progression.
- Progression-Free Survival (PFS):** Time from first dose to progression per RECIST 1.1 or death.
- Time To Response (TTR):** Time from first dose to first documented objective response.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring via HCP assessment, laboratory abnormalities, abnormal ECG parameters, clinical observations, and vital signs evaluation at every cycle.

Serious Adverse Events (SAEs): Required to be reported to the sponsor and regulatory authorities within 24 hours of site

awareness. **Data Monitoring Committee (DMC):** Independent external DMC monitoring safety across the sub-studies.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for efficacy outcomes; Safety Set for participants receiving at least one dose. **Sample Size Justification:** Targeted enrollment of 110 participants sized per sub-study (ranging from $N=20$ to $N=60$) to provide adequate precision for estimating the ORR and characterization of the safety profile. **Handling of Missing Data:** Missing data for tumor response will typically be counted as non-responders for the ORR endpoint; time-to-event endpoints will be right-censored.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Scheduled onsite and remote monitoring visits to ensure protocol adherence and data integrity. **Audit Trail:** Full documentation of eCRF entries, data clarification forms, and centralized radiological reading verifications.

11. Study Identifiers & Governance

Primary ID: D798MC00002 **Posting ID:** 892150010426638666 **Registry Number:** NCT06535607 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** eVOLVE-02 **Official Regulatory Title:** A Phase II, Multi-Center Study to Evaluate the Efficacy and Safety of Volrustomig as Monotherapy or in Combination With Anti-cancer Agents in Participants With Advanced/Metastatic Solid Tumors

12. Clinical Framework (Solid Tumors Specific)

Primary Conditions: Cervical Cancer, Head and Neck Squamous Cell Carcinoma (HNSCC) **Diagnosis Threshold:** Histologically or cytologically documented advanced/metastatic disease not amenable to curative therapy. **Medical Methodology:** PD-1/CTLA-4 bispecific antibody immune-checkpoint blockade +/- systemic chemotherapy (Cisplatin, Carboplatin, Paclitaxel, 5-FU). **Optimization Target:** Maximal tumor regression (ORR) and extension of Progression-Free Survival (PFS).

13. Study Procedures & Methodology

Management Pathway: Standard oncology treatment pathways coordinated with investigational immuno-oncology protocols. **Education Protocol (HCP):** Site initiation visits and regular protocol training regarding immune-related adverse events (irAE) management. **Education Protocol (Patient):** Informed consent process, reporting of immune-related side effects, and adherence tracking. **Quality Audit Mechanism:** Routine sponsor audits and Independent Central Review (ICR) for all radiological endpoints.

14. Observation & Follow-Up Schedule

Total Duration: Up to an average of 3 years (inclusive of treatment and follow-up) **Onsite Visit Cadence:** Aligned with treatment cycles (e.g., Q3W or Q4W depending on the specific chemotherapy backbone). **Remote Monitoring Frequency:** Standard ongoing clinical oversight. **Checklist Review Interval:** Tumor imaging assessments performed every 8 to 12 weeks.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	03-Apr-2026				
Clinical Operations Lead	[Name]	_____	03-Apr-2026		Lead		
Statistician	[Name]	_____	03-Apr-2026				